



Clinical Practice Guideline

Preoperative care

Reference #: NPG55

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to pre-operative care. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Staff Safety	☒
Other:	☐		

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1. Preoperative Preparation including Consent

1.1. Legislative Requirements

For patients aged over 18, undergoing non-urgent treatment, consent must be obtained from a third party, in circumstances where:

- The patients mental, intellectual, or conscious state does not permit them to provide valid consent.
- The patient is under the influence of alcohol or drugs, including premedication.
- An advanced health directive does not exist.

Where a patient is unable to give valid consent there is a defined treatment hierarchy that outlines the persons who are eligible to provide legal consent on the patients' behalf for treatment to proceed. For further information refer to: WA Department of Health Hierarchy of Treatment Decision Makers

[WA Hierarchy of Treatment Decision -Makers](#) (refer to appendix 1)

If there is no person who can give consent to treatment, a **Guardianship Order** is to be sought urgently.

In an emergency, where treatment is necessary to save a persons' life or prevent further serious injury and a person is incapable of giving valid consent, treatment may be provided without consent. This must be clearly documented in the patients' medical record.

For further information of legislation pertinent to Consent please refer to: WA Department of Health Consent to Treatment Policy MP 0175/22

[Consent to Treatment Policy](#)

1.2. Medical Requirements

Procedures performed under anaesthesia (general, regional, local and sedation) require the patient's written consent for treatment.

It is the responsibility of the treating Medical team to obtain consent.

Consent must be captured in writing on the SCGOPHCG Consent to Treatment (Form A). No abbreviations are to be used and the correct procedure, site and side are to be clearly documented. A Creutzfeldt-Jakob Disease (CJD) Risk Assessment form must be completed by Medical staff for pre-operative patients undergoing Neurosurgery (involving brain, pituitary, dura mater, spinal cord cranial and dorsal root ganglia, or the olfactory epithelium), and Ophthalmology surgery (involving retina and or optic nerve).

For further information refer to the SCGOPHCG Consent to Treatment Policy

[SCGOPHCG Consent to Treatment Policy](#)

1.3. Safety Information

Document on the perioperative record any allergies or hypersensitivities i.e. medications, foods, surgical tapes, or surgical preparations (such as Povidone-iodine, Chlorhexidine).

If patients have a latex allergy, please notify the theatre coordinator as soon as practicable.

RED patient identification bands and a RED theatre cap (applied in holding bay) are to be used for patients with documented allergies.

An “Adverse Drug Reaction” sticker must be placed on the perioperative record, medication charts, anticoagulation chart and fluid therapy chart for known drug allergies and hypersensitivities.

All items of metal are to be removed (e.g.: Jewellery, dermal body piercing, hairclips). Any items that cannot be removed are to be covered with adhesive tape except for Medi-alert bracelets. **Medi-alert bracelets must be left in situ and must not be taped.**

A wedding band may be left insitu except for patients undergoing:

- breast
- wrist
- hand
- cardiac or thoracic surgery,
- orthopaedic procedures to upper limbs that may cause swelling in the fingers
- finger surgery, all rings must be removed from the operative side.

A “Language Alert” label must be attached to the perioperative record or documented on perioperative checklist if they cannot understand English.

HCG/Pregnancy testing

OPH only:

- All female patients under 50 years or who continue to menstruate require HCG/pregnancy testing pre-operatively.
- Any Gynaecological surgery or surgical intervention with radiological imaging requires an HCG/pregnancy test prior to surgery. The date of Last Menstrual Period (LMP) and HCG/Pregnancy test result are to be detailed on the Perioperative Nursing Care Plan/checklist.

SCGH only:

- The pregnancy status of all female patients of childbearing age must be ascertained on admission. A pregnancy test is offered if the pregnancy status is unknown, and the outcome must be documented on the perioperative record including if a patient refuses a test. Surgeon must be notified of the outcome prior to administering a general anaesthetic especially if the procedure requires radiological imaging.

OPH only: Diabetic Patients – Patient Alert form is completed: Patient alert sticker affixed in the left column, detailed description such as: Diabetes Mellitus Type 1, DM Type 2, list of medication and/or insulin.

1.4.Nursing Management

Consent:

- Only when all aspects of consent are satisfied can the patient be given a pre-medication.
- Patients who do not speak English must have an appropriate interpreter available to them when the consent is:
 - Obtained
 - Checked prior to the procedure

Please contact the Language Services Office, between 8.30am and 5.00pm, Monday to Friday on extension 74698 or WA Interpreter Service 9451 6944. After normal office hours the Translating and Interpreting Service (T.I.S) may be contacted directly on 6211 5200 as this is a 24-hour service nationwide and out of hours Auslan Interpreting can be arranged on 0433 756 436.

Patients undergoing local anaesthetic require a three-hour booking for an interpreter. The interpreter is to accompany the patient whilst in theatre. Full theatre attire is to be worn by the interpreter.

Elimination:

- Assess and record the patient's bowel status pre-operatively on the Observation and response chart and/ or Bowel management chart. If the patient requires gastrointestinal lavage pre-operatively, document the patient's result on the Perioperative Record
- The patient must empty their bladder prior to surgery. Record the patient's last void on the fluid balance chart and on the Perioperative Record prior to transfer to theatre.
- If possible, perform post void bladder scan for Cardiothoracic patients and record post void bladder scan result on peri operative check list, and handover to theatre staff.

Skin Integrity:

- Assess patient's skin integrity and document pressure injury risk including applying prophylactic dressings as required.
- Document any skin problems and comment on pressure areas on the Perioperative Record. Anticipate the patient's post-operative pressure care needs and if required, arrange a pressure reducing / relieving mattress.
- Instruct or assist the patient to remove eye make-up. It is preferred that there is at least one clean digit (finger) available 'free of nail varnish and or an Acrylic nail' (or variation of)
- Ensure all wounds are covered and documented.

Documentation:

- Document relevant past medical history, the patients current mental state on the Perioperative Record
- Ensure assessment of the patient's risk for venous thromboembolism (VTE) is completed. Commence VTE prophylaxis as per Venous Thrombo-Embolism (VTE) SCGOPHCG Prophylaxis Clinical Practice Guideline i.e., administer medication as charted and apply Graduated Compression Stockings (GCS) or an Intermittent Pneumatic Compression as prescribed/ documented.
[SCGOPHCG Clinical Practice Guideline VTE Prophylaxis](#)
- Record patient's preoperative vital signs, temperature and weight on the observation and response chart as close to the time of giving the pre-medication as possible.
- Document patient self-administration of pre-operative medication e.g., nasal Mupirocin ointment, where required.
- Document completion of pre-operative chlorhexidine shower (i.e., 5 x chlorhexidine shower for elective hip/ knee surgery and Cardiothoracic surgery) on the Peri operative Chart
- Pregnancy status +/- HCG/Pregnancy test result, status unknown or patient refused must be documented on the perioperative record for all female patients of childbearing age and the surgeon notified.

Theatre Attire:

- Assist the patient into the following theatre attire. (If the patient has concerns regarding any aspect of the theatre attire, contact theatre / holding bay staff, as allowances can

be made in some circumstances). Theatre attire includes:

- Short sleeved gown open at the back.
- Paper theatre underpants
- Paper theatre cap (acquired in holding bay).
- Ensure the bed linen is clean.

1.5. Procedural Information

Procedure

1. Ensure the patient has two legible identification bands in situ. These should be on an arm and a leg, but not on a limb designated for surgery. (If this is not practical, an identification band placed on the left and right wrists or both ankles are acceptable).
2. **To assist with communication, hearing aids may be left in situ.** A labelled receptacle (denture cup or similar item) is to accompany the patient for safekeeping in case the hearing aid is to be removed intra-operatively.
3. Glasses or contact lenses:
 - SCGH: must be removed prior to transfer to theatre. Document their location on the Perioperative Record.
 - OPH: glasses are to be sent with patient, Label case with a patient ID sticker.
4. Document all other prostheses insitu (e.g., cardiac pacemaker, infusaport, artificial eye).
5. **Dentures may remain in situ** unless specified by the anaesthetist. If the patient chooses to wear their dentures to theatre, a **labelled denture cup** is to accompany the patient in case they are removed during intubation.
6. Ensure additional equipment that is required is available and sent with the patient (e.g., cervical soft collars, sandbags, splints, abduction pillows, Peritoneal Dialysis equipment, CPAP equipment etc.). If the patient requires CPAP, please ensure the settings of the machine have been checked as correct and send all relevant equipment with the machine. Label all equipment.
7. Theatre staff must be notified prior to transfer to holding bay if the patient is on contact, droplet or airborne precautions or weighs 120kg or more.
8. When the patient is ready for theatre, the nurse preparing the patient is to complete and sign the Perioperative Record.

2. Pre-Operative Skin Preparation and Body Hair Removal

2.1 Safety Information

Any abnormal skin irritation, abrasions, or signs of infection on or near the surgical site must be documented and the surgical team notified.

Razors **MUST NOT** be used for removal of body hair. Hospital issued surgical clippers are the only device to be used. Ensure clippers maintain continuous movement near the patients' skin to prevent friction and superficial burns.³

2.2 Nursing Management

- The type of surgery, operation site and specific surgeon preferences determines the extent of hair removal and skin disinfection used.
- If unsure check with the medical staff or surgical ward for surgeon preferences or contact the Floor Coordinator in theatre.
- Hair removal with surgical clippers¹ should be conducted as close as practical to the

time of surgery, preferably less than 2 hours prior to surgery.

- The patient should shower / bathe and wash hair using pre-operative body wash (e.g., Chlorhexidine 2 or 4%),^{2,3} on the morning of the procedure.
- Please refer to: SCGOPHCG Clinical Practice Guideline Mupirocin Nasal Ointment for Decolonisation for pre procedure duration of body wash for Cardiothoracic surgery and Orthopaedic Joint Surgery
[SCGOPHCG Guideline Mupirocin Nasal Ointment for Decolonisation](#)
- Do not apply creams, deodorants, or perfumes etc. prior to surgery.²

Recommended Surgical Preparation

Abdominal

- Ensure umbilicus is clean for ALL abdominal surgery.
- Clip as per diagrams or refer to surgical preference in ward areas.

Neurosurgical

- No surgical clips are required for patients undergoing back surgery.
- Clipping for patients undergoing craniotomy are performed in theatre.
- Refer to the *SCGOPHCG Clinical Practice Guideline Mupirocin Nasal Ointment for Decolonisation Guideline*, for details on preoperative use of 2% mupirocin nasal ointment and chlorhexidine 2% or 4% body wash.

Orthopaedic

- For orthopaedic surgery procedures involving implantation of prosthetic material:
- refer to [SCGOPHCG Guideline Mupirocin Nasal Ointment for Decolonisation](#) for details on preoperative use of 2% mupirocin nasal ointment and chlorhexidine 2% or 4% body wash.
- Note: For any emergency surgery where the full decolonisation regimen cannot be completed prior to the operation, each therapy (mupirocin ointment and chlorhexidine wash) should be initiated as soon as possible before surgery. This treatment is to be continued following the procedure to complete the full Five-day decolonisation regimen.
- Clip operative limb only if excessively hairy

Plastics

- Mammoplasty will require axillary clip.
- Skin graft site / Free flap site require clip only if excessively hairy.
- Free flap patients-to have whole body pre warmed for at least one hour preoperatively using warming device¹¹ (Not required for ENT free flap patients)

Check with surgical staff / Plastic CNS DECT # 73707 for consultant preferences.

Vascular

- Varicose Veins]
- Embolectomy] Clip only if hairy (clip groin area if full
- Vein graft]
- Aortic / femoral graft / abdominal Aortic aneurysm
 - Nipple to pubis including visible pubic hair and bilateral groin.
- Below knee amputation
 - Clip only if excessively hairy
 - Mid-thigh to ankle of affected limb
- Above knee amputation

- Clip only if excessively hairy – affected limb and pubic clip (on affected side only)

General

- a) Oesophagectomy – Ivor Lewis
 - Anterior neck to pubis + thoracotomy clip-on right-hand side.
- b) Mastectomy
 - Axillary clip to operative side
- c) Thyroidectomy
 - Clip neck area only if excessively hairy
- d) Laparoscopic Surgery
 - Check with the surgical team if clipping is required as the patient may progress to open surgery
- e) Open laparotomy - Surgery
 - Nipple to pubis

Cardiothoracic

- As per diagram 8 and 9, plus for cardiothoracic surgery procedures refer [SCGOPHCG Guideline Mupirocin Nasal Ointment for Decolonisation](#)
- Note: For any emergency surgery where the full decolonisation regimen cannot be completed prior to the operation, each therapy (mupirocin ointment and chlorhexidine wash) should be initiated as soon as possible before surgery. This treatment is to be continued following the procedure to complete the full five-day decolonisation regimen.

2.3 Procedural Information

Requirements

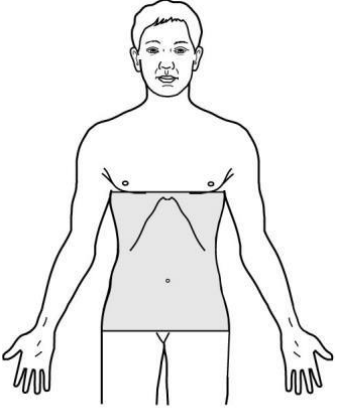
- Surgical clipping device
- Surgical tape
- Personal Protective Equipment (i.e., Nonsterile gloves)
- Scissors (optional)
- Tissues
- Personal Protective Equipment (Gloves)

Procedure ²

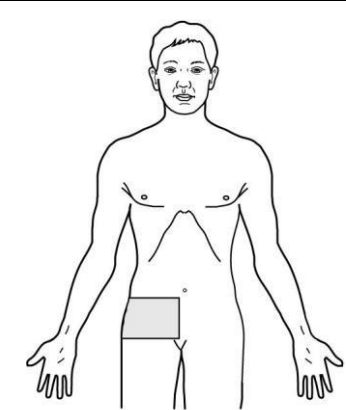
1. Check patient identity and operative site against consent form.
2. Assess the surgical site and surrounding skin before skin preparation and ensure the patient's skin is clean and dry.
3. Perform hand hygiene.
4. If necessary, cut hair prior to clipping.
5. If using surgical clippers follow natural body curves clipping against the direction of the hair growth.
6. In problem areas stretch the skin taut. DO NOT angle head of clipper into the skin.
7. Use the surgical tape to remove any loose hair from around the surgical site.
8. Discard clipper blade head.
9. Wipe the clipper device with combined detergent/ disinfectant wipe.
10. Perform hand hygiene.
11. Document on the Perioperative Record.

DIAGRAMATICAL GUIDE FOR PREOPERATIVE CLIPPING

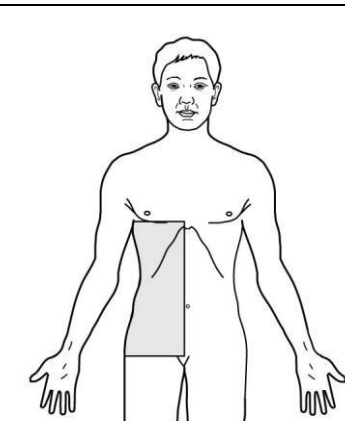
1.

	<u>Mid Axilla to Mid Axilla</u> • Hiatus hernia repair • Bowel resection • Cystectomy • Gastrectomy • Pancreatectomy • Splenectomy • Anterior resection • Liver transplant • Urethro lithotomy • Abdominal-perineal resection • Uretero-vaginal fistula closure • Vesico-vaginal fistula closure
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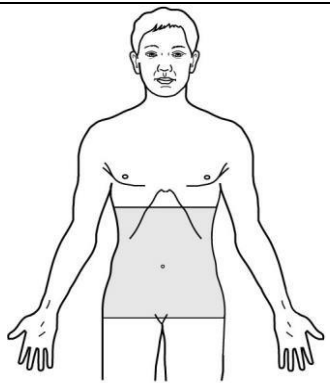
2.

	<u>Umbilicus to pubis</u> Midline to superior iliac crest • Open Appendicectomy
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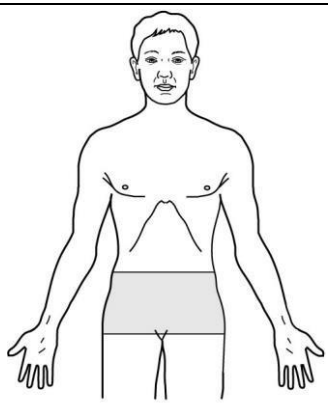
3.

	<u>Nipple line to pubis</u> Anterior midline to posterior midline (left to right) • Lumbar sympathectomy • Ureterolithotomy (upper 2/3 ureter) • Cholecystectomy • Nephrectomy • Pyeloplasty • Adrenalectomy • Thoracotomy (plus under arm clip)
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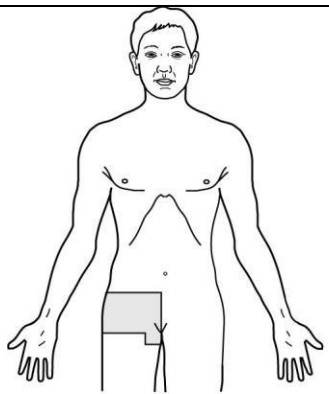
4.

	<u>Rib Cage to pubis</u> Mid axilla to mid axilla (including visible pubic hair but not perineal) • Re-implantation of ureters • Ureterolithotomy (lower 1/3 ureter)
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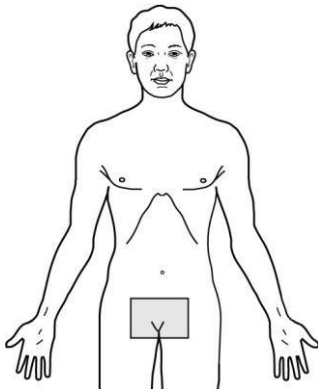
5

	<u>Umbilicus to pubis</u> Including visible pubic hair • Combined repair (bladder neck elevation and anterior plus posterior colporrhaphy) • Penile prosthesis – inflatable • Umbilical hernia surgery • Open prostatectomy • Renal transplant
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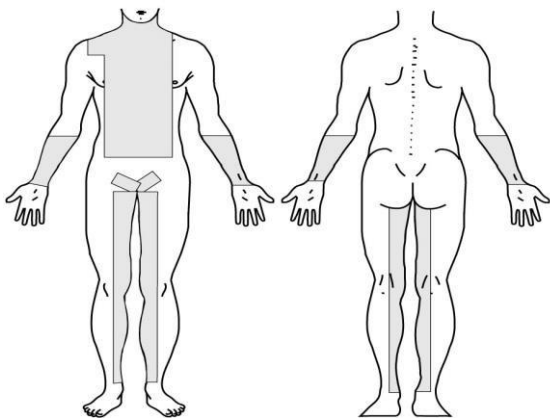
6.

	<u>Umbilicus to pubis</u> Anterior midline to (left or right) • Inguinal hernia surgery • Femoral hernia surgery
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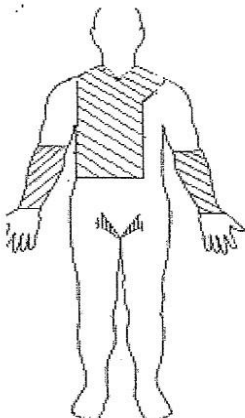
7

	<u>Bilateral groin clip and visible pubic hair</u> • Orchidectomy – if inguinal approach, clip appropriate side as well • Vasectomy • Hydrocelectomy • Epididymectomy
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8

	<u>Cardiac Surgery</u> NOTE: For cardiac valve repair/replacement surgery, complete bilateral groin clip but omit leg clip
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9

	<u>Minimally Invasive Mitral Valve Surgery</u> Access is via small right thoracotomy with multiple access sites on anterior and lateral chest The sternum is also to be clipped in case surgery has to be converted to a median sternotomy
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Fasting

2.4 Medical Requirements

Fasting is required for all types of anaesthesia (local, regional, general) unless the surgeon or anaesthetist specifies / documents otherwise. This is to protect the patient from possible regurgitation and aspiration of gastric contents.^{8,9,10}

However, patients should not be fasted for prolonged periods of time before their surgical procedure. Prolonged fasting can result in increased gastric volume and decreased gastric pH which is the opposite of the intent of fasting, as well as perioperative thirst, hunger, anxiety, and nausea. It also increases the risk of hypovolemia and haemodynamic instability, and a heightened surgical stress response resulting in unstable serum glucose.^{8,10}

Clinical outcomes for patients are improved when perioperative clear fluids (200mls) per hour are continued until the patient is sent to theatre. This 'Sip Til Send' protocol (compared to prolonged fasting) improves^{8,9,10,13}:

- Patient comfort
- Gastric emptying and pH
- Peripheral cannulation
- Post-operative recovery

2.5 Safety Information

Diabetic patients, commence monitoring blood sugar levels **hourly at the time of the first missed meal** and prior to escorting the patient to theatre. Document the most recent BSL on the Perioperative Record and include the diabetic sheet in the medical record sent to theatre. Please refer to [SCGOPHCG Insulin: Adult Variable Rate Intravenous](#) for indications to commence Intravenous insulin therapy for patients who are fasting.

2.6 Nursing Management

Patients prescribed Glucagon-like peptide-1 receptor agonists (GLP-1RAs) such as Semaglutide (Ozempic, Wegovy), Liraglutide (Victoza, Saxenda), Dulaglutide (Trulicity) and Tirzepatide (Mounjaro) have different fasting recommendations due to delayed gastric emptying.

Before surgery, patients on GLP-1RAs are to be directed to follow the recommended fasting status:

Clear Fluids only (refer to appendix 2) for the full day before surgery

- **Ensure it is clearly documented on the pre op checklist that patient is prescribed GLP-1RAs**
Refer to Peri-operative Medication Management Guideline for medication management
- Enter Clear Fluids as diet type into the Automated Menu System (AMS)

AM operating session	Clear fluids for the full day before surgery 24:00 – 24:00	Stop clear fluids but continue to drink water the night before at 24:00 until 06:00 (125mls or half a cup each hour)	From 06:00 stop drinking water and start fasting NBM
PM operating session	Clear fluids for the full day before surgery 06:00 – 06:00	Stop clear fluids but continue to drink water from 06:00 until 11:00 (125mls or half a cup each hour)	From 11:00 stop drinking water and start fasting NBM
Emergency theatre if you are unable to have the 24 hours clear fluids lead in time		Continue to drink water (125mls or half a cup each hour)	From 2 hours before scheduled surgery stop drinking water and start fasting NBM

Sip Til Send protocol for all other surgical patients:

Before surgery, patients are to be directed to follow the recommended fasting status (Sip til Send protocol):

- **no food for a minimum of 6 hours**
- **Preoperative clear fluids (refer to appendix 3) totalling no more than 200mls per hour** up until the time theatres send for the patient.
- Enter 'Sip til Send (STS)' as diet type into the Automated Menu System (AMS) for patients commencing Sip til Send protocol.
- Refer to Theatre Management System (TMS) for scheduled AM and PM theatre lists (guide only). Liaise with theatre coordinator for more accurate time
 - OPH extension 78087
 - SCGH extension 77305

For procedures using sedation please refer to area specific/ procedural fasting guidelines or contact the procedural areas for direction on clear fluid restrictions

AM Operating Session

Morning surgery, patients are to be directed to follow the recommended fasting status:

- No food from 02:00
- Continue preoperative clear fluids only, from 02:00 totalling no more than 200mls per hour until the time theatre sends for the patient.

PM Operating Session

Afternoon surgery, patients are to be directed to follow the recommended fasting status:

- A light early morning breakfast before 07:00 (e.g., 1 cup of fluid and 1 piece of toast), no food from 07:00
- Continue preoperative clear fluids only, from 07:00 totalling no more than 200mls per hour until the time theatre sends for the patient.

All Day Theatre Session

For all day theatre sessions, patients are directed to follow the recommended fasting status:

- No food from 02:00
- Continue preoperative clear fluids only, from 02:00 totalling no more than 200mls per hour until the time theatre sends for the patient.

Emergency Theatre

If surgery is scheduled to occur within 24 hours:

- Refer to Theatre Management System (TMS) or liaise with theatre coordinator for more accurate time.
- No food for a minimum of 6 hours.
- Continue preoperative clear fluids totalling no more than 200mls per hour until the time theatre send for the patient.
- Liaise with medical team to establish hydration status and consider Intravenous fluids if required. If medical team unavailable, contact anaesthetist for specific instructions.

ICU Patients

For patients meeting any of the following criteria, stop enteral feeding six hours prior to procedure:

- Non-intubated patients
- Patients with a non-cuffed tracheostomy tube
- Planned extubation, reintubation or airway manipulation.
- Planned prone positioning.
- Planned abdominal surgery.
- Upper GI endoscopy or equivalent (e.g., ERCP)

Otherwise:

- Continue enteral tube feeding at current rate until the patient is transferred to OT (or other procedural area)
- On transfer, cease enteral tube feeds and place the tube on gravity drainage.
- Suction the oral cavity and posterior pharynx.
- If the patient is receiving insulin, adjust as required.

3. Documentation

3.1 Medical Requirements

Surgical procedures performed under anaesthesia (general, regional and local require the patient's written consent. It is the responsibility of the treating medical team to obtain consent.

3.2 Safety Information

Ensure addressograph labels are correct. Ensure at least two full sheets of labels are supplied when the patient is transferred to theatre.

3.3 Nursing Management

Ensure Fluid intake is accurately recorded on the fluid balance chart

All current medical, nursing, and old medical records are to accompany the patient. Ensure all appropriate documents, results and requests mentioned below, are available and clipped to the front of the patient's file to allow for easy access.

Table 1. Documentation

Compulsory Documentation	Additional Documentation as appropriate
• Perioperative Record/checklist • Anaesthetic record • Consent Form • Pressure Injury Risk Assessment • Medication chart • Fluid orders (excluding cardiothoracic) • Fluid balance chart (excluding cardiothoracic) • Observation response chart • Anaesthetic consent form	• Diabetic chart • Eye treatment drug chart • Consent to donate/receive bone products • Neurovascular chart • Wound management plan • Peripheral Vascular insertion chart • Clinical pathway • Bladder washout chart • ICC drainage chart • CJD Risk assessment form • Anticoagulant chart

Request/Reports/Results as appropriate

- Pathology/histology requests (if required)
- ECG (if ordered)
- Cardiac Catheter report
- Private imaging (e.g., MRI) relevant to the procedure

4. Medications

4.1 Medical Requirements

The **Anaesthetist** or treating team is to specify which medications the patient is to have before surgery. If no medications are specified, **all routine medications are to be given**, which includes the patient's routine analgesia. Please check if unsure.

4.2 Safety Information

Please refer to **Peri-operative Medication Management Guideline** for information on medication in the pre and perioperative period. The final decision on medication management should be agreed upon by the Anaesthetist and Surgeon.

Check the front and inside of the medication chart for any premedication orders, including orders for pre-operative heparin and graduated compression stockings.

Only when all aspects of consent are satisfied can the patient be given a pre-medication.

4.3 Nursing Management

While fasting, any oral medication is to be given taking into consideration Sip Til Send protocol

Request the patient to attempt to void prior to administering the pre-medication. Advise the patient to remain in bed, raise the bed rails (with consent) and ensure the nurse call bell is within reach.

5. Transfer of Care / Escorting the Patient to Theatre

5.1 Safety Information

All points on the Perioperative Record are checked. Any variances are noted and communicated to the holding bay nurse (theatre coordinator OPH) or PACU (theatre nurse OPH) nurse after hours.

5.2 Nursing Management

For SCGH specific information:

- Patients being transferred to theatre require a completed Perioperative Record and are to be accompanied by a nurse escort. The nurse escort is required to provide a verbal clinical handover using iSoBAR principle and is to remain with the patient until the holding bay nurse (during hours) or the PACU nurse (after hours) has checked all aspects of the Perioperative Record and patient's preparation.
- The nurse escorting the patient is to sign the Perioperative Record.
- Patients (excluding Emergency Surgical urgency Category [ESUC 1]) in the

Emergency Department are transferred to theatre only on approval by the Duty Anaesthetist and Theatre Nurse Shift Coordinator.

For OPH specific information

- Patients being transferred to theatre require a completed Perioperative Checklist and are transported to theatre without a nurse escort unless clinically indicated. Please refer to SCGOPHCG Hospital Policy Clinical Handover – Communicating for Safety.

5.3 Procedural Information

Theatre Crosscheck ⁵

For SCGH specific information:

- The holding bay nurse or the PACU nurse (after hours) and the escorting nurse check the two identification bands are accurate.
- The holding bay nurse or the PACU nurse (after hours) completing the review signs the Perioperative Record.

For OPH specific information

- The OPH Anaesthetic Nurse will check with the patient that the two identification bands are accurate.
- The OPH Anaesthetic nurse completing the review signs the perioperative checklist.

Theatre Check on Transfer into the Operating Suite

For SCGH specific information:

- The operating theatre nurse collecting the patient is responsible for completing a surgical safety sign in. This must be done in the presence of anaesthetist and anaesthetic technician (if utilised for case) PRIOR to transfer to the operating room.
- The operating theatre nurse is to sign the Perioperative Record, before transferring the patient into the operating room.

For OPH specific information:

- The surgical safety sign in, is completed by either: Anaesthetic nurse, theatre nurse or medical staff within the theatre prior to procedure starting.
- The surgical safety checklist is signed to confirm check completed.

Related Documents

Policy:

- WA Department of Health Consent to Treatment Policy 2023
- SCGOPHCG Consent to Treatment Policy
- NMHS Clinical Documentation Policy
- SCGOPHCG Infection Prevention and Control Multi-Resistant Organism (MRO) Policy
- NMHS Patient Identification and Procedure Matching Policy
- SCGOPHCG Patient Identification Policy
- SCGH Procedure Matching Policy
- OPH Procedure Matching Policy
- WA Department of Health Language Services Policy 2017
- SCGOPHCG Antimicrobial Stewardship (AMS) Policy
- SCGOPHCG Mupirocin Nasal Ointment for Decolonisation Guideline
- SCGOPHCG Medication Management Standard and Procedure
- SCGOPHCG Peri-operative medication Management
- SCGOPHCG Clinical Practice Guideline VTE Prophylaxis

References

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12. Consent to Treatment Policy for the WA Health System 2023 Policy Framework MP 0175/22
13. Acknowledgement – the Preoperative Oral Fluids Sip Til Send Business Rule – Prince of Wales/Sydney-Sydney Eye Hospitals and Health Services June 2023

Authorisation & Version Control

Policy Sponsor	Clinical Practice Guideline Subcommittee				
Policy Contact	SCGOPHCG Policy Officer				
Date First Issued	September 1999	Last Reviewed:	June 2025	Review Date:	July 2027
Version No.	8.1				
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Endorsed by	Clinical Practice Guideline Subcommittee			Date:	28/07/2025
Standards Applicable	NSQHS Standards (Version 2): 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1 – WA Hierarchy of Treatment Decision - Makers



Government of Western Australia
Department of Health

Adult

WA Hierarchy of Treatment Decision-Makers

Where an Advance Health Directive does not exist or does not cover the treatment decision required, the health professional must obtain a decision for non-urgent treatment from the first person in the hierarchy who:

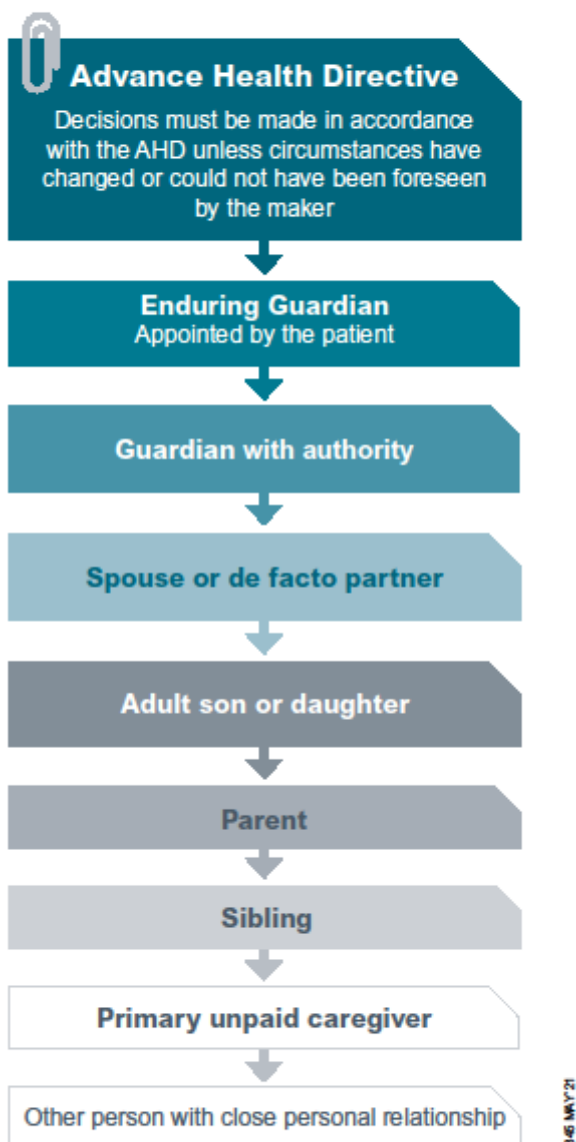
- is 18 years of age or older
- has full legal capacity
- is willing and available to make a decision

As a last resort if no one according to the hierarchy is available or willing, refer to the State Administration Tribunal

Adapted from the Office of the Public Advocate

Produced by the Patient Safety and Clinical Quality
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PSQ-013846 MAY'21

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Appendix 2 – Oral Table –Clear Fluids (for patients on GLP-1RAs)

Allowed	Not Allowed
• Normal medications (unless contraindicated) • Water • Ice Cubes/chips • Plain Jelly, sorbet with no fruit pieces • Clear soup e.g. stock powder (beef, chicken, vegetable) made with water • Apple Juice (clear, pulp free) • Black tea or coffee • Commercial rehydration or electrolyte drinks such as <i>Dex</i>™, <i>Hydralyte</i>™, <i>Gastrolyte</i>™, and <i>Powerade</i>™ • Resource fruit beverage such as <i>Fortijuice</i> or <i>Resource Ultra, Recover</i> • Clear fat free nutrition supplement (available at ward level) • Clear carbonated drinks (eg lemonade) • Citrus or Raspberry Cordial • Ice blocks/icy poles provided it is a clear fluid when in liquid form (i.e. can be seen through it when held up to light) • Sugar or artificial sweetener added to drink • Plain boiled lollies or sweets	• Milk (cow, goat, almond etc) • Anything dairy – eg skim milk, formula, yoghurt, Yakult™, watered-down milk, vanilla flavoured milks, ice cream • Starch or corn-starch • Bone broth, beef-extract or beef-tea • Any protein drinks • Anything with fruit pulp or vegetable fibres – eg “real” or freshly pressed/crushed apple, coconut, pineapple or other fruit juices • Orange or tomato juice • Coca cola • Anything given to “help get medication down” – eg Nutella, peanut butter, bread, yoghurt – no matter how small • Alcohol • Whilst chewing gum is technically allowed up until theatre, it should not be encouraged due to the risk of it becoming an inhaled foreign body if the patient forgets or chooses not to disclose it to the anaesthetist.
NB: Patients with diabetes usually require diet versions of the above allowed clear fluids.	
NO FOOD PRODUCTS IN THE 24 HOURS PRIOR TO INDUCTION OF ANAESTHESIA FOR PATIENTS ON GLP-1RAs	

Appendix 3 – Oral Table – Preoperative Clear Fluids (Sip til Send protocol)

Allowed totalling no more than 200mls per hour	Not Allowed
• Normal medications (unless contraindicated) • Water • Ice Cubes/chips • Apple Juice (clear, pulp free) • Black tea or coffee • Commercial rehydration or electrolyte drinks • such as <i>Dex</i>™, <i>Hydralyte</i>™, <i>Gastrolyte</i>™, and <i>Powerade</i>™ • Clear carbonated drinks (eg lemonade) • Cordial • Ice blocks/icy poles provided it is a clear fluid when in liquid form (i.e. can be seen through it when held up to light) • Sugar or artificial sweetener added to drink	• Anything with protein, fat or fibre • Thickened fluids • Milk (cow, goat, almond etc) • Anything dairy – eg skim milk, formula, yoghurt, Yakult™, watered-down milk, vanilla flavoured milks, ice cream • Lollies and sweets, even if only “sucking” • Starch or corn-starch • Bone broth, beef-extract or beef-tea • Jelly (contains gelatin – a protein) • Any protein drinks • Anything with fruit pulp or vegetable fibres – eg “real” or freshly pressed/crushed apple, coconut, pineapple or other fruit juices • Any fluid that is “cloudy” • Orange Juice • Coca cola • Anything given to “help get medication down” – eg Nutella, peanut butter, bread, yoghurt – no matter how small • Alcohol • Whilst chewing gum is technically allowed up until theatre, it should not be encouraged due to the risk of it becoming an inhaled foreign body if the patient forgets or chooses not to disclose it to the anaesthetist.
NB: Patients with diabetes usually require diet versions of the above allowed clear fluids.	
NO FOOD PRODUCTS IN THE SIX (6) HOURS PRIOR TO INDUCTION OF ANAESTHESIA	

Adapted from the Preoperative Oral Fluids Sip Til S Business Rule – Prince of Wales/Sydney-Sydney Eye Hospitals and Health Services